

# Standard Operating Procedure

## Blood and Blood Component Transfusion Administration

«Hospital Name» — Supports COP.8.d, COP.8.e, COP.8.f (Stop-Work)

|                       |                                                     |                         |                      |
|-----------------------|-----------------------------------------------------|-------------------------|----------------------|
| <b>SOP No.</b>        | [Hospital to define]                                | <b>Version</b>          | 1.0                  |
| <b>Effective Date</b> | _____                                               | <b>Review Frequency</b> | Annual, or on change |
| <b>Prepared By</b>    | Quality Coordinator Name: _____ Signature: _____    |                         |                      |
| <b>Approved By</b>    | Medical Superintendent Name: _____ Signature: _____ |                         |                      |

## 1. Purpose

This SOP describes how «Hospital Name» administers blood and blood components safely — correct testing, correct identification, correct monitoring, and a fast, correct response to any adverse reaction.

## 2. Scope

Applies to every transfusion of whole blood or blood components, in every clinical area.

## 3. Responsibility

**Treating Doctor:** orders the transfusion with a clear, evidence-based indication.

**Nursing Staff:** performs identity verification, administers, and monitors the transfusion.

**Blood Bank Staff:** completes pre-transfusion testing and releases only cleared units.

## 4. Procedure

### 4.1 Before the transfusion

1. Confirm the indication for transfusion against the hospital's evidence-based criteria before ordering.
2. Obtain informed consent from the patient or attendant.
3. Confirm the unit has cleared TTI testing and Red Cell Serology, and is correctly labelled — never issue or accept an unlabelled or untested unit.

### 4.2 Identity and order verification

1. Verify the patient's identity using two identifiers, cross-checked against the unit label, by two staff members independently — this double-check is mandatory, every time.
2. Cross-check the patient's blood group against the unit twice before starting the transfusion.

### 4.3 During the transfusion

1. Maintain the cold chain until the moment the unit is started.
2. Monitor the patient closely, especially in the first 15 minutes, when most acute reactions present.
3. If any adverse reaction is suspected — fever, chills, rash, breathlessness, back pain — stop the transfusion immediately, keep the line open with normal saline, and call the treating doctor.

## 4.4 After a suspected reaction

1. Report every suspected adverse reaction, evaluate it, and document the evaluation.
2. Apply CAPA for the reaction, and report to the Haemovigilance Programme of India.

## 5. Stop-Work Authority

Do not transfuse a unit without completed pre-transfusion testing, or without two-identifier verification cross-checked against the unit.

Do not continue a transfusion once an adverse reaction is suspected — stop immediately.

Raising an identity mismatch or a testing gap before transfusion begins is not a disciplinary matter.

## 6. Records

Supported by Records 5, 8, 9, 15, and 16 of the COP.8 Required Records pack.

## 7. Related Documents

- HCO\_COP\_8\_v2\_REWRITE\_DRAFT — COP.8 policy
- SOP — Blood Donor Selection and Collection (companion SOP for blood bank operations)

## 8. Review

Reviewed annually, and immediately after any serious transfusion reaction or mismatch near-miss.

## 9. References

- Association for the Advancement of Blood & Biotherapies (AABB). Fundamental Standards for Blood Collection and Transfusion (2nd edition).  
<https://www.aabb.org/standards-accreditation/standards/fundamental-standards-for-blood-collection-and-transfusion> — a free standards document designed for lower- and middle-income country settings, covering donor qualification, collection, processing, storage, and pre-transfusion testing.